



**HKU
Med**

LKS Faculty of Medicine
School of Clinical Medicine
香港大學臨床醫學學院

BACHELOR OF MEDICINE AND BACHELOR OF SURGERY



ASSISTANT INTERNSHIP

Obstetrics and Gynaecology

MBBS V/VI – Assistant Internship

Obstetrics & Gynaecology

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Assistant Internship Student Handbook 2025

Obstetrics and Gynaecology

1. Aim and objectives

Aim:

Students should be able to become competent interns and to acquire competencies to the level of a general practitioner

In general:

- equip themselves with knowledge and skills to participate in health care of women, including fertility regulation, prevention and early detection of diseases by screening;
- be aware of the effects of disorders of the female genital tract, and of pregnancy, delivery and puerperium on health in women;
- understand the patient-centred care for obstetrical and gynaecological disorders;
- respect patient's rights, privacy and confidentiality;
- be an effective member of a health care team and to participate in quality assurance, clinical audit and peer review; and
- develop self-learning and critical appraisal ability and to acknowledge the importance of continuing medical education

Objectives:

Students should be able to:

(1) Human Biology in Health and Disease

- a) understand normal physiology of the female genital tract and reproduction system and normal physiology of pregnancy
- b) understand the pathophysiology and clinical presentation of common gynaecological and obstetrical disorders

(2) Professional Skills: Diagnostic, Problem Solving, Effective Communication and clinical Management

- a) diagnosis common gynaecological or obstetrical problems by proper history taking and clinical examination
- b) understand the use and limitations of investigative tests involved in O&G
- c) communicate to patients and other health care workers
- d) diagnose and manage common O&G disorders
- e) recognize and institute correct initial management in O&G emergencies

(3) Population Health, Health Services, Economics and Policy

- a) understand the current O&G care provision systems in Hong Kong e.g. antenatal care system, women's health check and screening system, family planning services, etc.
- b) be aware of public health policies related to O&G

(4) Medical Ethics, Professional Attitudes and Behaviour

- a) understand ethical issues involved in various areas in O&G e.g. assisted conception techniques, prenatal diagnosis, preterm deliveries, care for both mother and baby in a pregnant woman, issues on termination of pregnancy, end of life issues in gynaecological malignancies, etc.
- b) be aware the psychosexual issues involved in various gynaecological and obstetrical problems
- c) demonstrate respect for patient's privacy and confidentiality
- d) develop the concept of continued medical education, self-learning and critical appraisal
- e) participate in quality assurance, clinical audit and peer-review
- f) be aware of risk management and legal issues

2. Objectives of different types of Obstetrics & Gynaecology teaching activities

At the end of the Assistant Internship, students should be able to:

2.1 Orientation and online orientation lectures

- a) know the objectives of learning in Obstetrics & Gynaecology and the logistics of the time-table
- b) obtain the essential information to enable them to follow the rest of the Assistant Internship teaching

2.2 Ward teaching

- a) diagnose common Obstetrical & Gynaecological conditions and understand how doctors determine the management of the patient
- b) understand the progress and management of patients

2.3 Gynaecological Ward, EPAC and Operating Theatre Attachment Week

- a) Management of patients suffering from common Gynaecological conditions requiring inpatient care
- b) Management of patients suffering from early pregnancy complications in outpatient settings
- c) Practise presentation skill and professional exchange of opinion
- d) Exposure to common operations in General Gynaecology

2.4 Outpatient clinic attendance

- a) take a detailed O&G history
- b) demonstrate O&G related clinical skills
- c) communicate with patients their diseases and what should be done, and to communicate with other members of the health care team
- d) demonstrate the etiquette of being a doctor

2.5 Women's Health Seminar on Family Planning Methods

- a) discuss the principles of birth control options ~~and menopause care~~

2.6 Workshop

- a) demonstrate O&G physical examinations and practice relevant procedure
- b) take a detailed history on various Obstetrics and Gynaecology problems
- c) counsel women on various Obstetrics and Gynaecology conditions
- d) discuss issues on medical ethics and law in Obstetrics & Gynaecology

2.7 Clinical PBL

- a) experience clinical management of patient through team work and self-learning
- b) present clinical findings and discuss relevant issues
- c) critically appraise information and management

2.8 Labour ward attachment

- a) participate in conducting normal and abnormal labour and delivery
- b) understand intrapartum and immediate post-partum care and complications and their management

2.9 Critical appraisal of journal

- a) appraise literature

2.10 Audit Meeting (online video):

- a) understand quality assurance, clinical audit and peer review

3. List of Core Topics to be covered within the Assistant

3.1 Obstetrics

Part I – Antenatal Care

- Physiology of pregnancy
- Antenatal screening & prenatal diagnosis
- Antenatal care (including minor ailments of pregnancy, prolonged pregnancy, assessment of fetal well-being), pre-pregnancy and pregnancy counseling
- Fetal growth assessment (including discrepancy between size and date)
- Multiple pregnancies
- Antepartum haemorrhage
- Malpresentation
- Medical disorders (other than HT and DM), including drug use in pregnancy
- Hypertension in pregnancy
- Gestational diabetes

Part II – Labour & Delivery

- Preterm labour (including premature rupture of membranes)
- Management of labour (normal, abnormal labour and obstetrics analgesia)
- Operative deliveries and Lower Segment Caesarean Section (including previous LSCS)
- Postpartum haemorrhage

Part III – Puerperium

- Puerperium and lactation

3.2 Gynaecology

Part I – General Gynaecology

- Menstrual disorders (including abnormal vaginal bleeding, dysmenorrhoea)
- Endometriosis
- Genital tract infection (including vaginal discharge and pelvic inflammatory disease)
- Early pregnancy complication
- Urinary incontinence and uterine prolapse
- Climacteric

Part II – Reproductive Medicine and Endocrinology

- Subfertility
- Fertility Regulation

Part III – Neoplasia

- Benign ovarian tumours (including abdominal / pelvic mass)
- Benign uterine tumours (fibroid, adenomyosis, polyps)
- Genital tract cancer
- Cervical cancer prevention

4. Clinical Skills

At the end of the Assistant Internship, the students should be able to:

4.1 General Obstetrics and Gynaecology

- provide initial management of gynaecological and obstetrical emergencies
- recognize the need for referral and to write a referral letter or medical report
- critically appraise patient management with relevance to available resources

4.2 Gynaecology

- elicit a full gynaecological history and to communicate with the patient and other health care personnel
- perform gynaecological examination and to detect gross abnormality
- use a bi-valve speculum to expose the cervix, and take a cervical smear
- diagnose common gynaecological abnormalities
- communicate with patients the relevant information about their abnormalities and explain to them what will need to be done

4.3 Obstetrics

- carry out an adequate antenatal examination and to recognize abnormalities
- diagnose pregnancy
- recognize common antenatal complications of pregnancy
- recognize the onset of labour and the ability to recognize deviations from normal labour and puerperium
- conduct antenatal care and postnatal care as a member of a health care team
- advise patients on matters concerning their health and family planning and breast feeding

5. Activities *(All teaching activities are compulsory.)*

5.1 Orientation

- Introduction
- Normal labour and delivery
- Labour ward orientation and visit

Online videos are available at the HKU Moodle website:

HKU Moodle (<http://moodle.hku.hk>) → log in → My Courses

→ MBBS Bachelor of Medicine and Bachelor of Surgery Programme (MBBS) curriculum

→ Assistant Internship → O&G → Online Videos / Web-based Learning Modules

Topic
Analgesia and anaesthesia for obstetrical patients
Antenatal care: Objectives, Theories and Practice
Antenatal examination (video)
Breastfeeding
Gynaecological emergencies
HKWC Medical Student Clerkship Clipboard
Principles of operative deliveries and previous Caesarean Section
Common instruments in Gynaecology
Gynaecological Examination (Part 1) - Introduction
Gynaecological Examination (Part 2) – Pelvic examination
Perinatal Meeting
Caesarean Section
<i>(Surgery) Scrubbing</i>
<i>(Surgery) Gowning and Gloving</i>

Students are expected to go through all the online videos within the first week of the Assistant Internship to obtain the essential information to enable them to follow the rest of the Assistant Internship teaching. It takes about 5.5 hours to finish all the online videos.

5.2 Obstetrical Case Discussions / PBL

Types of Cases:

1. Gestational diabetes
2. Multiple pregnancy
3. Uterine size smaller than date
4. Malpresentation
5. Puerperal problems (mandatory)
6. Antepartum haemorrhage
7. Previous Caesarean Section

5.3 Gynaecological Case Discussions / PBL

Type of Cases:

1. Endometriosis / adenomyosis
2. Subfertility
3. Genital prolapse / urinary incontinence
4. Pelvic mass / ovarian cyst
5. Postmenopausal bleeding
6. Cervical screening / abnormal cervical cytology (mandatory)
7. Female sterilization / contraception
8. Bleeding in early pregnancy
9. Vaginal discharge / Pelvic Inflammatory Disease
10. Gestational trophoblastic diseases / hydatidiform mole
11. Dysfunctional uterine bleeding
12. Ectopic pregnancy
13. Uterine mass and menorrhagia
14. Amenorrhoea (mandatory)

5.4 Obstetric Clinical Activities

For obstetrics, each clinically admitted patient is assigned to one student at the time of admission. Students are required to follow up their patients while they are in hospital. Students will be assigned to the first stage and labour ward on a roster basis. They have to stay in these two wards throughout the whole period of their assignment. They have to choose two patients whom they must continuously observe throughout the whole period of labour and this is known as “close observation”. The aim is to help students understand the progress of labour and the monitoring of the progress of labour. Operating theatre attendance may take place during the labour ward roster, or when the assigned patient has an elective operation (Caesarean Section).

5.5 Labour Ward Attachment

1. Please revisit the video of “antenatal examination”, get familiarized with stages of labour and the departmental manual on “management of labour” before the attachment.
2. There are designed labour ward attachment sessions in HA hospitals (QMH/ KWH/ PYNEH/ QEH/ UCH), two of which to take place in QMH (8am – 8pm). For the QMH labour ward attachment session, the students are expected to “shadow” the on-duty intern or medical officer in K9S antenatal ward and K9N labour ward when they admit patient, perform assessment or procedures 4pm-8pm.
3. Please write two detailed reports of close observation of labour and complete the partogram. Students are expected to discuss one of them with the birth attendant (midwife or doctor).
4. Students are required to observe and assist deliveries. The number of assisted deliveries should be at least 5, which should be vaginal/ instrumental/ Caesarean section. They will need to ask for patient’s consent and they need to be supervised by a doctor or a midwife.

“Delivery” means any delivery, including normal delivery, instrumental delivery, vaginal breech delivery, Caesarean section. Assisting in delivery does not mean just observation. The student assisting the delivery should perform the following procedures, if possible:

- Scrub and gown up
 - Clean and drape the patient, in vaginal deliveries
 - Draw local anaesthetics if applicable
 - Clamp and cut the cord in vaginal deliveries, if the baby’s condition is stable
 - Dry the baby and hold the baby for the mother to see, if the baby’s condition is stable, in vaginal deliveries
 - Assist in the repair of the episiotomy or tears
 - Clean the vulva after delivery or the repair of the perineum
 - Be the 2nd assistant in Caesarean sections
 - Clean and weigh the placenta, examine the placenta
5. Please complete and ask the supervisors to sign the labour ward attachment session in the student logbook. In the table for “Encounters or Procedures in labour ward attachment”, there are core observation items (in bold type), which are mandatory. Students are encouraged to document their observations during the attachment.

5.6 Gynaecological Ward, EPAC and Operating Theatre Attachment Week

The doctors in K5S are divided into 2 teams. One of the teams will be operating on Tuesday to Friday. Apart from patients with operations, the team will also look after other patients previously managed by the team. The other team will be seeing patients in the clinics and all other patients admitted. The two teams swap their duties at the end of the week.

Each student will have one week of ward attachment. The week will generally start on Monday and finished on Friday. During the week, students will be assigned to work with one of the teams in K5S. Please refer to a specific timetable for the roster and the team each student is assigned to. Each patient will have one student following her through during her stay in the ward. They are expected to observe/assist doctors performing examination or procedures during their period of attachment. They are also expected to join the other activities of the Division, including the **Consultant Rounds** starting at 8 am every Monday morning at K5S MO room, the **Division Meetings** at 5 pm every Monday at K509 and the **Preoperative Rounds** at 5 pm every Thursday at K5S MO room

During the days when the team is doing mainly operating duties, the students will need to see all patients of the team before the gynaecologist does the ward round, usually at around 8 am. They are expected to present their patient's clinical information during the ward round. They will go to the **operating theatre** on Tuesday, Wednesday or Friday. They are also expected to join and present in the evening round.

During the days when the team is doing mainly ward and clinic duties, students are also expected to join the morning and evening ward rounds. They are also expected to stay in the ward during their 'free' time so that they will admit other patients assigned to their team.

Each student should go to 1-2 sessions of **Early Pregnancy Assessment Clinic (EPAC)** during his or her attachment to the ward team. EPAC runs every Monday to Friday at 10:00am in K5N. Some of the patients are admitted to the ward in the afternoon after being seen in the EPAC in the morning. If the students have seen them in the clinic, they are expected to look after these patients after admission.

During the **Tuesday General Gynaecology Ward Teaching** and **Thursday Grand Rounds**, students are expected to present the patients they follow.

Students may also be assigned to go to some clinics during their ward attachment. Please refer to the specific timetable.

5.7 Gynaecological Oncology teaching round

Students are expected to clerk patients in K13S for oncology teaching round. The patients in K13S are allocated by admission. Students should check the admission book for their allocated patients. Students are expected to clerk the patients allocated to them and follow up on their progress. The students may be asked to present these patients during the oncology teaching round. Students having their "Gynaecological Ward, EPAC and Operating Theatre Attachment Week" can be exempted from clerking patients in K13S, but they can still do so if they wish to.

5.8 Out-patient Clinics

Students will be assigned to sessions of Outpatient Clinics (OPD) and primary care. They will interview, examine and advise patients during these clinic sessions.

5.9 The clinical and teaching activities include

Obstetrics & Gynaecology (whole class)

-
- Obstetric case discussion, PBL in ward
- Gynaecology case discussion, PBL in ward
- Abdominal and Vaginal Examination
- Audit meeting
- Women's Health

Obstetrics Group

- Obstetrics grand / teaching round
- Antenatal Clinic / Postnatal Clinic
- Combined Endocrine Clinic
- Day Centre (Obs)
- Labour ward roster (including obstetric ward attachment and operating theatre attendance)

Gynaecology Group

- General Gynaecological Outpatient Clinics
- Birth Control Clinic
- Menopause Clinic
- Pre-admission Clinic
- Subfertility Clinic
- General Gynaecological Ward, EPAC and Operating Theatre Attachment Week
- General Gynaecological grand / teaching round
- Gynaecological Oncology teaching round

Workshop

- Physical Examination
- O&G Communication 1
- O&G Communication 2

Critical Appraisal of Journal

5.10 Web-based Learning:

Web-based learning module is available at the HKU Moodle website:

HKU Moodle (<http://moodle.hku.hk>) → log in → My Courses

→ MBBS Bachelor of Medicine and Bachelor of Surgery Programme (MBBS) curriculum

→ Assistant Internship → O&G → Online Videos / Web-based Learning Modules

Topic
Introduction to colposcopy
Postnatal mood problems
Preterm labour and preterm premature rupture of membranes
Puerperal fever
Antenatal screening and prenatal diagnosis of thalassaemia
Female urinary incontinence and urodynamics
Postmenopausal Bleeding
Prenatal screening for Down syndrome
A woman with oligomenorrhoea
Amenorrhoea
Molar Pregnancy - Gestational Trophoblastic Disease
Use of Ultrasonography in Obstetrics and Gynaecology

6. Log Book

Students are required to keep a logbook, which will record the diagnosis and procedures of all patients that they have seen, activities taken during the Assistant Internship, and tutor's assessment. The students' logbooks will be examined by their tutors from time to time and at the end of the Assistant Internship.

Workshop

There will be 3 workshops (three-hour each) on the 1st day and two Fridays during the Assistant Internship for the whole group. Students are required to study for both obstetrics and gynaecology topics irrespective of whether they start with obstetrics or gynaecology rotation in order to actively participate in the workshops. There will be 1 to 4 tutors. The tutors will give guidance to students during the workshop and give marks for the group work.

Group Work

Reflective moment in O&G

Ethical issues in O&G – case discussions

Single Best Answer (SBA) Question writing

7. Undergraduate Assessment

1. End-of-Rotation Clinical Competence Assessment (CCA): **40%**
2. Continuous Assessment: **20%** (including performance in workshops, assessment of logbook, group work, SBA question writing and online videos/modules)
3. Written Examination: **40%** (at Final MBBS Examination, single-best-answer MCQ and EMQ)

The Obstetrics and Gynaecology *Assistant Internship* clinical competence assessment will take the form of a 12-station OSCE assessment (8 minutes per station) and clinical skill examinations held during the last 1 or 2 OPD assignments. The stations are listed as follows: -

Station	Theme	Objective
1	History taking	To assess the skill in taking a full and proper obstetrical or gynaecological history
2	Management and counselling	To assess the communication skill and knowledge on the management issues and decision making in primary care obstetrics / gynaecology
3	Communication with colleague	To assess the communication skill with colleague and knowledge on the management issues and decision making in primary care obstetrics / gynaecology
4	Viva on obstetrics / gynaecology management	To assess the knowledge on the management of an obstetric / gynaecologic clinical scenario
5-8	Pre-pregnancy / Antenatal / Intrapartum / Postpartum care	To assess the knowledge on clinical management of common pre-pregnancy, antenatal, intrapartum or postpartum conditions
9-12	General gynaecology / Gynaecological oncology / Reproductive medicine / Urogynaecology	To assess the knowledge on clinical management of common conditions in general gynaecology, gynaecological oncology, reproductive medicine or urogynaecology

Stations 1 to 4 will be interactive stations and may be involving interaction with a surrogate patient. Stations 5 to 12 will be non-interactive stations and may include interpretation of patients' charts / investigation results / clinical objects.

Students must pass the end of rotation Clinical Competence Assessment in order to pass the Final MBBS Examination in Obstetrics and Gynaecology. The students who fail in the Assistant Internship Clinical Competence Assessment have to re-sit the Clinical Competence Assessment before the Final MBBS Examination in March/April of the following year. Failing this Clinical Competence Assessment will result in the failure of the Final MBBS Examination in Obstetrics and Gynaecology.

60% is considered as pass mark.

8. Remedial

Special slow learners or students who are absent because of illness or other reasons will be given the chance to do remedial Assistant Internship.

9. Student Evaluation of the Course

Students are expected to participate in the evaluation of the Obstetrics and Gynaecology teaching. There are two aspects of evaluation. The first consists of an end-of-rotation evaluation of all the teaching activities against the objectives, and the students' perception of their learning. The second is an evaluation of teachers. The Department will consider the comments/suggestions and to improve the course. During the Assistant Internship, students will also be invited to attend two Staff-Students Consultative Meetings to report on any problem that occurs and their comments regarding the Assistant Internship. The teachers will also tell the students their comments on their performance as a group.

10. Teaching Timetable & Student Grouping List

To be uploaded to the HKU Moodle System prior to each rotation.

11. Contact List

Specialty Co-ordinator	Professor Karen Chan / Professor Siew-Fei Ngu	
Teaching Timetable /	Miss Phyllis Li	(2255 4567)
General Enquiries	Ms Hazel Yuen	(2255 4647)

12. Recommended Texts

(It is advisable for students to read more than on recommended texts):

<u>Book Name</u>	<u>Edited by</u>
1. Fundamentals of Obstetrics and Gynaecology	Derek Llewellyn-Jones
2. Essential Obstetrics and Gynaecology	E.M. Symonds
3. Obstetrics and the newborn	Norman A. Beischer
4. Obstetrics (Ten Teachers)	Sir Stanley Clayton, etc.
5. Gynaecology (Ten Teachers)	Sir Stanley Clayton, etc.
6. Obstetrics Illustrated	Alistair Miller & Robert Callander
7. Gynaecology Illustrated	A.D.T. Govan, Colin Hodge & Robert Callander
8. Dewhurst's Textbook of Obstetrics and Gynaecology for Postgraduates	C.R. Whilfield
9. Practical Obstetrics and Gynaecology	Pak-chung Ho
Print edition: https://hkupress.hku.hk/Books/Subject/medicine-health/POG2	
E-book:	
Amazon kindle https://www.amazon.com/dp/B0CTMMG17W	
Google Play https://play.google.com/store/books/details?id=d5jxEAAQBAJ	

Other References:

1. Departmental Manual, Department of Obstetrics and Gynaecology, The University of Hong Kong
HKU Moodle (<http://moodle.hku.hk>) → log in → My Courses → MBBS Bachelor of Medicine and Bachelor of Surgery Programme (MBBS) curriculum → Assistant Internship → O&G → Departmental Manual
2. The patient information sheets for common obstetric and gynaecological procedures are available in our Departmental Website:
<https://obsgyn.med.hku.hk/en/Patient-Information/Patient-Information>
3. Family health service web page: breastfeeding policy and practical skills video:
<http://www.fhs.gov.hk/english/breastfeeding/policy.html>
http://www.fhs.gov.hk/english/multimed/breastfeeding/bf_vid_c6p4_2.html
http://www.fhs.gov.hk/english/multimed/breastfeeding/bf_vid_c6p4_3.html
http://www.fhs.gov.hk/english/multimed/breastfeeding/bf_vid_c6p4_4.html
http://www.fhs.gov.hk/english/multimed/breastfeeding/bf_vid_c6p4_5.html

13. Code of Conduct

(a) Student's Rights

All students shall have the right to:

- enjoy a safe learning environment,
- express own opinions and feelings,
- use and access to school facilities, services, and resources,
- establish and join student societies, and
- acquire knowledge and skills leading to the award of a degree if the required standard is reached.

(b) Student's Responsibilities

- Be punctual and attend school activities prepared.
- Participate fully in the learning process.
- Complete assignments and comply with the timeline.
- Reach the academic standards required in the programme.
- Use proper language and observe dress code.
- Work and co-operate with peers and staff members (including office staff, healthcare staff).
- Refrain from aggressive and intimidating behaviour.
- Treat patients' information in confidentiality.

(c) Academic Misconduct

(*Academic dishonesty will constitute a disciplinary case, which may result in disciplinary action being taken on you)

- Cheating in examination.
- Plagiarism (please refer to section a below).
- Misrepresentation of data/research projects.
- Allowing another student to represent your work as his/her own.
- Forgery or alternation of official records or documents including attendance records or "doctor/nurse" status.
- Manifest attitude or behaviour which is detrimental or illegal, involving people in the direct context of teaching/learning activities *i.e.*, patients.

(d) Plagiarism

- Any unacknowledged use, as one's own, of the thoughts, writing, inventions or work of another person, whether or not such work has been published, will be an act of plagiarism and is a serious misconduct, which may result in disciplinary actions.
- For detailed information and consequences of plagiarism, please visit the University's website on plagiarism at <https://tl.hku.hk/plagiarism/>.

(e) Use of Teaching Materials

- Course materials are for teaching purposes and are not meant for circulation without prior approval from the Faculty/departments/individual teachers.
- Unauthorised photographing, recording and video-taping of teaching sessions and/or practical sessions and downloading of presentation files are strictly forbidden.
- Any unauthorised recording and/or downloading of teaching materials would constitute a disciplinary case.
- All online materials available on Moodle shall be the property of the University of Hong Kong by law, which shall not be retained, disclosed, reproduced, transferred or distributed, performed or displayed, in parts or in full, without explicit permission from the Faculty.
- Instructions of using/dealing with specimens and manikins should be adhered.

(f) Attendance

- Attend all teaching sessions as prescribed in the timetable.
- Sign the attendance sheet for each teaching session with your real name.
- Signing attendance for another person is considered forgery and it is a serious misconduct which may result in disciplinary actions.
- If students cannot attend a class because of illness, they should submit in writing an application for leave of absence to the Associate Dean (Teaching and Learning) via the Faculty Office together with a certificate signed by a registered medical practitioner.
- Students should submit the application for leave of absence and submit supporting document to the Faculty Office within 7 days of the session that they missed.
- For cases of absence on non-medical reasons, students should apply to the Associate Dean (Teaching and Learning) via the Faculty Office in writing at the earliest opportunity, stating the reasons for taking leave of absence. Applications will be considered individually. Permission for such leave shall be granted only in exceptional circumstances and shall not be granted by reason only that they have already made arrangements to be absent.

(g) Non-Academic Misconduct

(*Non-academic misconduct may result in disciplinary actions taken on you, e.g. severe reprimand and termination of student status)

- Manifest attitude or behaviour which is illegal or detrimental to people in the general context
- Verbal/physical abuses: endangers or threatens the health or safety of university members
- Harassment
- Disrupt/obstruct university activities or services
- Thievery
- Unauthorised conduct: unauthorized entry to and/or presence at, possession of, use of any university facilities/resources
- Causing damage/destruction to university prop

(h) Professional Attitude and Behaviour of a Medical Student

(i) Professional Attitude

- Demonstrate honesty and integrity
- Be prepared to admit deficiencies in knowledge, understanding or skills
- Be aware of own's strengths and weaknesses
- Seek help, guidance, support or advice to overcome difficulties
- Face mistakes or failings positively
- Be ready to accept fair and reliable criticism or evaluation from teachers, fellow schoolmates and/or patients
- Accept responsibilities and consequences for your own decisions and actions
- Treat all other students, teachers, and staff members with respect and dignity
- Speak and listen and respect the rights of others; Show tolerance and acceptance of all genders, ethnics, races, religions
- Show respect towards school property and resources
- Respect of the "patient": dead or alive, organ or whole, physical or mental issue.

(ii) Attire

- Dress properly on all learning occasions as a matter of respect to the teaching activity, your peers, the teacher, and yourself; whether these sessions are held in wards or not.
- For clinical teaching sessions, e.g., bedside teaching, outpatient clinics, clinical skills sessions, students are required to wear WHITE COAT with name badge and appropriate professional attire.
- Appropriate attire can inspire confidence of the patients in you as a healthcare professional, and also avoid being the vehicle of infection transmission (cleanliness).

(iii) Use of Cell Phone

- Keep them turned off or "silent mode" during class
- DO NOT talk on cell phone or send messages during class
- DO NOT send or receive test/exam materials (Serious Academic Misconduct)
- DO NOT take or distribute inappropriate digital photos.

(iv) Eating and Drinking in Class

- DO NOT eat or drink (except water) in all teaching venues

(v) Behaviour in Wards/Clinics

- Show courtesy and consideration for everyone (including patients, doctors, nurses, fellow students, and other staff) in the ward
- Consult the tutors/ ward managers/ nurses on duty in wards for advice before performing patient clerking
- Strictly follow infection control measures and guidelines as stipulated by the Faculty and teaching hospitals concerned

(vi) Patient Confidentiality

- DO NOT take patient records away from the wards/clinics, and DO NOT copy or save the records in any formats even for learning purpose
 - DO NOT disclose, in various means of oral or written format, by telephone or fax, or electronically, any medical information in connection with the management of a patient
- Students are reminded NOT to release any private information that they may have obtained during the course of their studies.

(vii) Patient Contact

- Patients have the right to decide what happens to their own bodies.
- Valid consent should be obtained before starting clinical enquiry or examination, and patients have a right to refuse.
- Conduct history taking or examination with due care to patient's dignity and comfort.
- Thank patient after completing the learning process.
- Restore the clothing or positioning of the patient to his/her original or desirable status.
- Put back the files/records properly.
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(vii) Chaperone Policy for Physical Examination

- Clinical chaperone is a person who has a role to witness and safeguard both doctor (student) and patient during physical examination.
- Female chaperone is required to be present for physical examination of female patients by male students.
- Students should provide clear explanation and seek the consent from the patient concerned before physical examination.
- The chaperone should have the knowledge to assess the appropriateness of the examination/procedure as well as the way in which they are done.
- Adequate "screening" during physical examination requiring patient body exposure, and the chaperone should be in the sight of the patient.

DO NOT release private information that you may have obtained during the course of your studies. (Note that this may be done unintentionally if you talk/discuss freely in public areas of the hospital/clinic*)**